# NCE Rapid Review Cheat Sheets

Five high-yield review sheets for the final weeks before the exam

**Free Reader Bonus #2 for NCE Exam Prep 2026–2027**

**Keystone Test Prep Team**

**NCE Rapid Review Cheat Sheets**

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Exam facts in this bonus match NCE Exam Prep 2026–2027, which draws on NBCC's official documents. Testing policies change. Always confirm rules, fees, and dates in the most recent official materials.

## How to Use These Sheets

These sheets pull the most tested facts from NCE Exam Prep 2026–2027 into one place. They are built for the Lock In phase of your study cycle and for the final seven days before the exam. Use them for quick review, not as your first introduction to a topic. When a line on a sheet does not make sense, go back to the matching chapter in the book and relearn it there.

- **Cover and recall.** Cover the right-hand column of a table and try to say the answer before you look.
- **Mark your misses.** Put a small dot next to any line you get wrong. Two dots means it belongs in your weak-spot tracker.
- **Review little and often.** Ten minutes a day with one sheet beats a two-hour session the night before.

# Sheet 1: The 2027 NCE at a Glance and Numbers to Know

| Feature | Current NCE (before July 1, 2027) | 2027 NCE (from July 1, 2027) |
|---|---|---|
| Total questions | 200 | 170 |
| Scored questions | 160 | 140 |
| Unscored field-test questions | 40 | 30 |
| Answer choices per question | 4 (A–D) | 3 |
| Time to answer questions | 225 minutes (3 hours 45 minutes) | 225 minutes (3 hours 45 minutes) |
| Break | One scheduled 15-minute break after question 100 | Optional 15-minute break halfway through |
| Review after the break | Online handbook: no return to first 100 questions after the break | No return to the submitted half after the break |
| Score type | Raw score (maximum 160) with a passing score that varies slightly by form | Scaled score from 100 to 500 |
| Passing score | Set by modified Angoff method and equating for each form | Scaled score of 360 on every form |
| Domain structure | 6 domains based on a 2019 job analysis | 6 new domains based on a work analysis finalized in October 2025 |
| Largest domains | Counseling Skills and Interventions (30%), Areas of Clinical Focus (29%) | Provision of Counseling Interventions (20%), Legal & Ethical Compliance (20%) |
| Domain feedback | Score report provided | Preliminary pass/fail plus general feedback at each domain level |
| Guessing penalty | None | None stated; answer every question |

## The six 2027 domains

| Domain | Weight |
|---|---|
| 1. Professional Development & Counselor Self-Awareness | 15% |
| 2. Intake & Assessment | 18% |
| 3. Treatment Planning & Continuity of Care | 15% |
| 4. Provision of Counseling Interventions | 20% |

| Domain | Weight |
|---|---|
| 5. Indirect Client Care | 12% |
| 6. Legal & Ethical Compliance | 20% |

## Numbers to know

| Number | What It Means |
|---|---|
| 68 / 95 / 99.7 percent | Scores within ±1, ±2, and ±3 standard deviations on a normal curve |
| Mean 0, SD 1 | z-score |
| Mean 50, SD 10 | T-score |
| Mean 100, SD 15 | Deviation IQ |
| Mean 5, SD about 2 | Stanine |
| 0.05 | Common alpha level (probability of a Type I error) |
| 0.2 / 0.5 / 0.8 | Small, medium, and large effect sizes (Cohen's d) |
| 2–3 / 4–5 / 6 or more | Mild, moderate, and severe substance use disorder (criteria met within 12 months) |
| 2 weeks | Minimum duration of a major depressive episode |
| 2 years (1 year for youth) | Persistent depressive disorder and cyclothymic disorder |
| 1 week / 4 days | Manic episode (or any duration with hospitalization) / hypomanic episode |
| 1 day to under 1 month | Brief psychotic disorder |
| 1 to under 6 months | Schizophreniform disorder |
| 6 months (1 month active) | Schizophrenia |
| 3 days to 1 month | Acute stress disorder after trauma; more than 1 month suggests PTSD |
| Within 3 months / within 6 months | Adjustment disorder onset after a stressor / resolution after the stressor ends |
| 12 months (6 months for youth) | Prolonged grief disorder, measured from the death |
| 6 months | Generalized anxiety disorder |
| 5 years | Minimum time before any romantic relationship with a former client may even be considered (ACA Code) |
| 1974 | FERPA |
| 1976 | Tarasoff decision (duty to protect) |
| 1996 | Jaffee v. Redmond (federal psychotherapist-patient privilege) |
| 2009 | HITECH Act |

# Sheet 2: Ethics and Legal Concepts

| Topic | Key Points |
|---|---|
| Ethical principles | Autonomy, nonmaleficence, beneficence, justice, fidelity, veracity |
| Decision making | Identify problem; apply code; determine dimensions (principles, literature, consult); generate options; consider consequences; evaluate (justice, publicity, universality); implement; document |
| Ethics vs. law conflict | Make commitment to code known, work to resolve; if unresolved, may follow the law; consult |
| Informed consent | Ongoing; understandable; includes procedures, risks, benefits, credentials, fees, confidentiality limits, records, technology, diagnosis implications, right to refuse |
| Minors | Parent or guardian usually consents; seek minor's assent; agree on confidentiality at outset; know state law |
| Confidentiality limits | Serious and foreseeable harm; abuse reporting; court order; client waiver; client-initiated legal action; treatment team, supervision, and payers (with notice) |
| Communicable disease | Disclosure to identifiable third parties at high risk permitted (not required) only if state law allows, after assessing client's intent |
| Duty to protect | Tarasoff (1976); serious danger to reasonably identifiable victim; state laws mandatory, permissive, or absent; consult and document |
| Privilege | Belongs to the client; Jaffee v. Redmond (1996) recognized federal psychotherapist-patient privilege |
| Mandated reporting | Reasonable suspicion of child abuse or neglect; many states include older and dependent adults; report personally; good-faith immunity |
| Subpoena vs. court order | Subpoena: respond, contact client, consult, assert privilege. Court order: generally comply; may request limits |
| Witness roles | Fact witness reports observations; expert witness gives opinions; do not combine treating and forensic roles |
| Civil commitment | Danger to self, danger to others, or grave disability; least restrictive alternative; due process; clear and convincing evidence (Addington v. Texas) |
| Sexual relationships | Prohibited with current clients and their partners or family; prohibited with former clients for 5 years and only if not exploitive after |
| Nonprofessional interactions | Only when potentially beneficial; document rationale and expected consequences |
| Values | Do not impose personal values; no values-based referral |
| Competence | Practice within training and supervised experience; get training and supervision for new specialties |
| Impairment | Monitor for impairment; seek help; limit practice if clients could be harmed |

| Topic | Key Points |
|---|---|
| Colleague misconduct | Informal resolution first when appropriate; report when unresolved or harm is substantial |
| Records | Accurate, timely, secure; retain per law; client access unless compelling evidence of harm; secure disposal |
| HIPAA | Privacy, Security, and Breach Notification Rules; covered entities and business associates; BAA; minimum necessary; psychotherapy notes protected; stricter state law governs |
| HITECH (2009) | Breach notification; increased penalties; EHR adoption |
| FERPA (1974) | Education records; rights transfer at 18 or postsecondary; sole possession notes exception |
| 42 CFR Part 2 | Extra protection for substance use disorder program records |
| ADA and Section 504 | Reasonable accommodations; effective communication; service animals (not emotional support animals) |
| NPI | 10-digit provider identifier; Type 1 individual, Type 2 organization |
| CAQH ProView | Credentialing data shared with health plans |
| Counseling Compact | Privilege to practice in member states with unencumbered home-state license; follow law where client is located |
| Telehealth | Verify identity and location; secure platform with BAA; reconnection and emergency plans; laws of counselor's and client's states |
| AI | Consent, privacy, BAA, accuracy review, bias awareness, human judgment, no AI crisis replacement |
| Billing | CPT 90791, 90832, 90834, 90837, 90839, 90840, 90846, 90847, 90853; no upcoding, billing no-shows to insurers, or false diagnoses |
| Liability | Occurrence vs. claims-made (tail coverage); malpractice = duty, breach, direct causation, damages |
| Marketing | Accurate advertising; no soliciting testimonials from clients or vulnerable people; separate personal and professional social media; do not confirm client status online |
| Emergency preparedness | Hazard assessment, life safety, communication, telehealth continuity, data backups, professional will, drills, regular review |

# Sheet 3: DSM-5-TR Diagnosis Comparisons

These tables focus on distinctions that are easy to confuse. See Chapter 5 for fuller descriptions.

## Mood disorders

| Feature | Major Depressive Disorder | Persistent Depressive Disorder | Bipolar I | Bipolar II | Cyclothymic Disorder |
|---|---|---|---|---|---|
| Minimum duration | 2 weeks | 2 years (1 year youth) | Mania 1 week or hospitalization | Hypomania 4 days plus major depressive episode | 2 years (1 year youth) |
| Mania | Never | Never | Required | Never | Never full episode |
| Hypomania | Never | Never | May occur | Required | Subthreshold symptoms |
| Major depressive episode | Required | May co-occur | Common, not required | Required | Never full episode |

## Trauma and stressor-related disorders

| Feature | Acute Stress Disorder | PTSD | Adjustment Disorder | Prolonged Grief Disorder |
|---|---|---|---|---|
| Trigger | Traumatic event | Traumatic event | Any identifiable stressor | Death of a close person |
| Timing | 3 days to 1 month after trauma | More than 1 month | Onset within 3 months; resolves within 6 months after stressor ends | At least 12 months after death (6 months for youth) |
| Key point | Short-term trauma response | Four symptom clusters | Distress or impairment beyond expected | Persistent, impairing yearning or preoccupation |

## Psychotic disorders

| Feature | Brief Psychotic Disorder | Schizophreniform | Schizophrenia | Schizoaffective | Delusional Disorder |
|---|---|---|---|---|---|
| Duration | 1 day to under 1 month | 1 to under 6 months | 6 months or more (1 month active) | Lifetime pattern | 1 month or more |
| Mood episode | Not central | Not central | Not central | Concurrent major mood episode plus 2 weeks of psychosis without mood episode | Not central |
| Return to baseline | Full return | Not required | Functional decline | Varies | Functioning largely intact |

## Anxiety and related disorders

| Disorder | Core Fear or Focus | Duration |
|---|---|---|
| Generalized anxiety disorder | Many everyday matters | 6 months |
| Panic disorder | Fear of more panic attacks | 1 month of worry or behavior change |
| Agoraphobia | Situations where escape is difficult | Typically 6 months |
| Social anxiety disorder | Negative evaluation by others | Typically 6 months |
| Specific phobia | Specific object or situation | Typically 6 months |
| Separation anxiety disorder | Separation from attachment figures | 4 weeks (youth); typically 6 months (adults) |
| Obsessive-compulsive disorder | Obsessions and compulsions | Time-consuming or distressing |
| Illness anxiety disorder | Having or getting a serious illness | 6 months |

## Disorders of childhood that are often confused

| Feature | ADHD | Oppositional Defiant Disorder | Conduct Disorder | DMDD |
|---|---|---|---|---|
| Core feature | Inattention and/or hyperactivity-impulsivity | Angry or irritable mood, defiance, vindictiveness | Violation of others' rights and major norms | Severe temper outbursts with persistent irritability |
| Timing | Symptoms before 12; 6 months | 6 months | 12 months (one symptom in past 6) | 12 months; onset before 10; diagnosed ages 6 to 18 |
| Settings | Two or more | May be limited to one | Varies | Two or more settings |

## Eating disorders

| Feature | Anorexia Nervosa | Bulimia Nervosa | Binge-Eating Disorder | ARFID |
|---|---|---|---|---|
| Weight | Significantly low | Usually normal or above | Often above, can vary | Nutritional deficiency or weight loss possible |
| Binge eating | Possible (binge/purge type) | Yes, at least weekly for 3 months | Yes, at least weekly for 3 months | No |
| Compensatory behaviors | Possible | Yes | No regular compensation | No |
| Body image disturbance | Yes | Self-worth tied to shape and weight | Distress about binges | No |

## Delirium versus major neurocognitive disorder versus depression in older adults

| Feature | Delirium | Major Neurocognitive Disorder | Depression |
|---|---|---|---|
| Onset | Sudden (hours to days) | Gradual | Weeks to months |

| Feature | Delirium | Major Neurocognitive Disorder | Depression |
|---|---|---|---|
| Course | Fluctuating | Progressive | Often worse in mornings; may respond to treatment |
| Attention | Markedly impaired | Usually intact early | Reduced effort, may say "I don't know" |
| Action | Urgent medical evaluation | Medical evaluation and support planning | Assessment and treatment of depression; medical evaluation |

## Substance use disorder severity

| Number of Criteria Met (within 12 months) | Severity |
|---|---|
| 2–3 | Mild |
| 4–5 | Moderate |
| 6 or more | Severe |

# Sheet 4: Statistics and Assessment Terms

| Term | Plain-Language Meaning |
|---|---|
| Nominal scale | Categories with names only (diagnosis type) |
| Ordinal scale | Ranked order without equal spacing (class rank) |
| Interval scale | Equal spacing, no true zero (Fahrenheit) |
| Ratio scale | Equal spacing with a true zero (number of sessions) |
| Mean | Average; pulled toward extreme scores |
| Median | Middle score |
| Mode | Most frequent score |
| Positive skew | Tail to the right; mean > median > mode; hard test |
| Negative skew | Tail to the left; mean < median < mode; easy test |
| Range | Highest minus lowest score |
| Variance | Average squared distance from the mean |
| Standard deviation | Typical distance from the mean |
| Normal curve | About 68% within ±1 SD, 95% within ±2 SD, 99.7% within ±3 SD |
| Percentile rank | Percent of norm group scoring at or below a score; not percent correct |
| z-score | Mean 0, SD 1 |
| T-score | Mean 50, SD 10 |
| Deviation IQ | Mean 100, SD 15 |
| Stanine | Nine bands; mean 5, SD about 2 |
| Correlation (r) | Strength and direction of a relationship, −1 to +1; not causation |
| Coefficient of determination | r squared; shared variance |
| Reliability | Consistency of scores |
| Test-retest reliability | Stability over time |
| Alternate forms reliability | Equivalence of two versions |
| Split-half reliability | Agreement of two halves; Spearman-Brown correction |
| Internal consistency | Items measure the same thing; Cronbach's alpha, KR-20 |
| Inter-rater reliability | Agreement between scorers; Cohen's kappa |
| Standard error of measurement | Expected spread of observed scores around the true score; ±1 SEM ≈ 68%, ±2 SEM ≈ 95% |
| Validity | Accuracy of score interpretations for a purpose |

| Term | Plain-Language Meaning |
|---|---|
| Content validity | Items cover the content domain |
| Concurrent validity | Relates to a criterion measured now |
| Predictive validity | Predicts a future criterion |
| Construct validity | Measures the theoretical trait; includes convergent and discriminant evidence |
| Face validity | Looks like it measures the trait; not true validity |
| Sensitivity | Correctly identifies people who have the condition |
| Specificity | Correctly rules out people who do not have the condition |
| Norm-referenced | Compared with a norm group |
| Criterion-referenced | Compared with a fixed standard |
| Ipsative | Compared with the person's own other scores |
| Speed vs. power test | Many easy items under time pressure vs. harder items with ample time |
| Independent variable | What the researcher manipulates or groups by |
| Dependent variable | The outcome measured |
| Internal validity | Confidence that the intervention caused the change |
| External validity | Generalizability of results |
| Type I error | False positive; rejecting a true null hypothesis |
| Type II error | False negative; missing a real effect |
| Alpha level | Probability of Type I error, often 0.05 |
| Power | Probability of detecting a real effect; increases with sample size |
| Effect size | Size of an effect; Cohen's d ≈ 0.2 small, 0.5 medium, 0.8 large |
| t-test | Compares two means |
| ANOVA | Compares three or more means |
| Chi-square | Compares frequencies in categories |
| Regression | Predicts an outcome from predictors |
| Formative evaluation | Feedback during a program to improve it |
| Summative evaluation | Judgment of overall effectiveness at the end |
| Needs assessment | Identifies gaps before designing a program |

## Sheet 5: Theorists and Theories

Use this chart for rapid review. Each row pairs a key figure with the theory, core concepts, and signature techniques most likely to appear on the NCE.

| Key Figure(s) | Theory or Model | Core Concepts | Signature Techniques or Terms |
|---|---|---|---|
| Sigmund Freud | Psychoanalysis | Id, ego, superego; unconscious; psychosexual stages; defense mechanisms | Free association, dream analysis, interpretation, transference |
| Carl Jung | Analytical psychology | Collective unconscious, archetypes, persona, shadow, individuation | Dream work, exploration of archetypes |
| Alfred Adler | Individual psychology | Social interest, inferiority, lifestyle, birth order, private logic | Early recollections, encouragement, acting as if, spitting in the soup |
| Carl Rogers | Person-centered | Actualizing tendency, conditions of worth, incongruence | Empathy, unconditional positive regard, congruence |
| Viktor Frankl | Logotherapy (existential) | Will to meaning, meaning in suffering | Paradoxical intention, dereflection |
| Rollo May, Irvin Yalom | Existential therapy | Death, freedom, isolation, meaninglessness | Exploring responsibility and meaning |
| Fritz and Laura Perls | Gestalt therapy | Here and now, awareness, unfinished business, contact boundaries | Empty chair, two-chair, exaggeration |
| Ivan Pavlov, John B. Watson | Classical conditioning | Learned associations between stimuli | Conditioning, extinction |
| B. F. Skinner | Operant conditioning | Reinforcement, punishment, schedules | Token economies, shaping |
| Joseph Wolpe | Behavior therapy | Reciprocal inhibition | Systematic desensitization |
| Albert Bandura | Social learning (social cognitive) theory | Modeling, self-efficacy, reciprocal determinism | Modeling, behavioral rehearsal |
| Albert Ellis | Rational emotive behavior therapy | ABC model, irrational beliefs, "musts," unconditional self-acceptance | Disputing (ABCDE) |

| Key Figure(s) | Theory or Model | Core Concepts | Signature Techniques or Terms |
|---|---|---|---|
| Aaron Beck | Cognitive therapy / CBT | Automatic thoughts, schemas, cognitive triad, distortions | Socratic questioning, thought records, behavioral experiments |
| Donald Meichenbaum | Cognitive behavior modification | Self-talk | Stress inoculation, self-instructional training |
| William Glasser, Robert Wubbolding | Reality therapy / choice theory | Five basic needs, quality world, responsibility | WDEP, planning |
| Steve de Shazer, Insoo Kim Berg | Solution-focused brief therapy | Exceptions, client strengths, small changes | Miracle, exception, scaling, coping questions |
| Michael White, David Epston | Narrative therapy | Problem-saturated stories, re-authoring | Externalizing, unique outcomes |
| William Miller, Stephen Rollnick | Motivational interviewing | Ambivalence, change talk, spirit of MI | OARS, rolling with discord, decisional balance |
| Marsha Linehan | Dialectical behavior therapy | Acceptance and change; biosocial theory | Mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness |
| Steven Hayes | Acceptance and commitment therapy | Psychological flexibility | Defusion, acceptance, values, committed action |
| Segal, Williams, Teasdale | Mindfulness-based cognitive therapy | Relapse prevention for recurrent depression | Mindfulness practices with cognitive principles |
| Cohen, Mannarino, Deblinger | Trauma-focused CBT | Child trauma with caregiver involvement | PRACTICE components |
| Patricia Resick | Cognitive processing therapy | Stuck points about trauma | Challenging beliefs; trauma themes |
| Edna Foa | Prolonged exposure | Emotional processing of trauma | Imaginal and in vivo exposure |
| Francine Shapiro | EMDR | Adaptive information processing | Bilateral stimulation, eight phases |
| Richard Schwartz | Internal family systems | Self and parts (managers, firefighters, exiles) | Parts work |
| Murray Bowen | Family systems (multigenerational) | Differentiation, triangles, emotional cutoff, multigenerational transmission | Genogram, detriangling |
| Salvador Minuchin | Structural family therapy | Subsystems, boundaries, hierarchy | Joining, enactments, restructuring |

| Key Figure(s) | Theory or Model | Core Concepts | Signature Techniques or Terms |
|---|---|---|---|
| Jay Haley, Cloé Madanes | Strategic family therapy | Power, hierarchy, attempted solutions | Directives, paradox, ordeals |
| Virginia Satir | Experiential / human validation | Self-esteem, communication stances | Family sculpting |
| Carl Whitaker | Symbolic-experiential therapy | Spontaneity, growth | Use of self, absurdity |
| Sue Johnson | Emotionally focused therapy | Attachment, negative interaction cycles | Accessing and reshaping emotions |
| John and Julie Gottman | Gottman method | Four Horsemen, Sound Relationship House | Gentle start-up, repair attempts |
| Irvin Yalom | Group psychotherapy | Therapeutic factors | Here-and-now process |
| Bruce Tuckman | Group development | Forming, storming, norming, performing, adjourning | Stage-matched leadership |
| Frank Parsons | Trait-and-factor career theory | Self-knowledge, knowledge of work, true reasoning | Matching |
| John Holland | Vocational personalities | RIASEC, congruence, consistency, differentiation, identity | Self-Directed Search |
| Donald Super | Life-span, life-space | Self-concept, life stages, life-career rainbow | Career maturity and adaptability |
| John Krumboltz | Social learning theory of career decision making | Genetic endowment, environment, learning experiences, task approach skills | Planned happenstance |
| Linda Gottfredson | Circumscription and compromise | Narrowing by sex type and prestige | Exploring eliminated options |
| Lent, Brown, Hackett | Social cognitive career theory | Self-efficacy, outcome expectations, goals | Addressing barriers and supports |
| Mark Savickas | Career construction | Life themes, career adaptability | Career story interview |
| Erik Erikson | Psychosocial development | Eight stages and crises | Stage-based understanding |
| Jean Piaget | Cognitive development | Schemas, assimilation, accommodation; four stages | Developmentally matched interventions |
| Lawrence Kohlberg | Moral development | Preconventional, conventional, postconventional | Moral dilemmas |
| Carol Gilligan | Ethic of care | Care-based moral reasoning | Relational focus |

| Key Figure(s) | Theory or Model | Core Concepts | Signature Techniques or Terms |
|---|---|---|---|
| John Bowlby, Mary Ainsworth | Attachment theory | Secure base; secure, avoidant, resistant, disorganized | Strange Situation |
| Urie Bronfenbrenner | Ecological systems | Micro, meso, exo, macro, chrono systems | Contextual assessment |
| Prochaska, DiClemente | Transtheoretical model | Stages of change | Stage-matched interventions |
| Marlatt, Gordon | Relapse prevention | High-risk situations, abstinence violation effect | Coping plans |
| Kübler-Ross; Worden; Stroebe and Schut | Grief models | Stages (not linear); tasks of mourning; dual process | Grief counseling |
| Gerald Caplan | Mental health consultation | Client- and consultee-centered, case and administrative | Consultation types |
| Janine Bernard | Discrimination model of supervision | Teacher, counselor, consultant roles | Intervention, conceptualization, personalization |
| Stoltenberg and colleagues | Integrated Developmental Model | Levels 1–3 of supervisee growth | Developmentally matched supervision |

## One Small Request

If NCE Exam Prep 2026–2027 has helped you, please consider leaving an honest review on Amazon. To find it, search for the book's title. Share your real opinion, whatever it is. Reviews help other counseling graduates decide whether the book fits how they study.